1. Aim:

To provide guidelines to the ward staff on how to discharge the patient .

2. Rationale:

- To ensure that the discharge of the patient takes place smoothly.
- To ensure patient safety, satisfaction & comfort during discharge.
- To reduce the time involved in planned discharge & physical discharge.
- To protect the hospital from medico legal issues involved in DAMA.

3. Procedure:

- Consultants write a discharge orders after the consultant finds the patient fit to get discharge.
- The resident doctor on duty put the discharge notes and then ward nurse checks the billing activity , updates it and then send it for billing and summary .
- Resident doctors check the discharge summary & sign on it & obtain the signature of the consultant also.

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- Return all required medications to the pharmacy .
- Remove all blank documents. Collect all pending investigation reports.
- If the patient is Pvt then handover all Original copies of investigations to the patient and keep photocopies in the IPD file.
- If the patient is TPA then handover all photocopies of investigations to the patient and keep Original copies in the IPD file.
- (In TPA case all originals are faxed to the TPA)
- If the patient is MLC then handover all photocopies of investigations to the patient and keep Original copies in the IPD file.
- After receiving a call from IP billing about the readiness of the bill, inform patient to clear all pending bills.
- After the full and final stamp is put , Director will sign on 3 bills, 2 receipts and occupancy.
- Ward staff records the bill no. and the receipt number (only after seeing the full and final and sign is done)
- Check for the hospital property i.e. linen, crockery etc.
- Handover the discharge summary to the patient and/ or relative only after checking the final settlement of the bill.
- Obtain signature of the person to whom you are handing over all the documents in the IP register & Report Acknowledgement Form along with their name & relationship.
- If there are any pending reports of the investigations, if the patient party requests then inform them that the reports would be couriered to them.

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- The on duty doctor will explain the discharge instructions to the patient &/ or relative & take a feedback.

- Relieve the patient. If required the patient would be transferred via wheelchair or stretcher.
- If required arrangement for transportation would be done through reception.

IN CASE OF MEDICO LEGAL CASE (MLC PATIENT DISCHARGE):

- Consultants put the discharge order after he finds the patient is fit to get discharge.

- Ward RMO prepare return MLC and send it to Dhantoli police station. If the patient is stable and getting discharge police sign on it and allow to discharge the patient BUT IF THE PATIENT IS DEATH THEN BODY IS HANDED OVER TO THE POLICE ONLY.

- Document the police officer's name buckle and name of police station.

- DO NOT HANDOVER any original documents to patient including original discharge summary, original printed reports, original X – Ray, C.T. scan, MRI films & CDs.

- The photocopy of printed reports to be handed over to the patient or his relative.

- Obtain signature of the person to whom you are handing over all the documents in the admission register along with date, their name & relationship.

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IN CASE THIRD PARTY ADMINISTRATOR (TPA PATIENT DISCHARGE):

- DO NOT HANDOVER any original documents to patient including original discharge summary, original printed reports, original X – Ray, C.T. scan, MRI films, ECGS & CDs.
- The photocopy/ XEROX of only printed reports to be handed over to the patient or his relative.
- Obtain signature of the person to whom you are handing over all the documents in the admission register along with their name & relationship.

IN CASE OF DAMA (DISCHARGE AGAINST MEDICAL ADVICE)

1. Procedure:

(FOLLOW THE POLICY AND PROCEDURE FOR NORMAL DISCHARGE PATIENT BUT FOLLOW THE ADDITIONAL INSTRUCTIONS GIVEN BELOW)

- Consultant put the discharge order after the patient expresses his/her wish to get DAMA.
- The consultant or RMO explain the patient/ relative about the condition of the patient in their own language.
- Obtain the consent for DAMA from patient./ relative.
- Obtain the High Risk consent from two relatives in case of life threatening condition.(NEGATIVE COUNSELLING)

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- Resident prepares the discharge summary stating this discharge as DISCHARGE AGAINST MEDICAL ADVICE.
- Send file for the discharge summary.
- RMO checks the discharge summary & sign the same & obtain the signature of the consultant on it.
- Handover the reports to the relatives(if its not MLC case)

Content of a typical DISCHARGE SUMMARY

Patient Name:

Registration No.:

Reason for admission:

Significant finding:

Investigation results:

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Procedures performed:

Final Diagnosis: :

Treatment given :

Patient condition at time of discharge:

Follow up advice :

Medication and other instruction :

The discharge summary should contain the following instruction

Emergency care is available 24 X 7 Please Contact :- 0712-6627900
Report back to the hospital immediately in case of malaise, fever and chills, localized redness, heat, swelling and pains, vomiting of blood, unconsciousness, convulsion, difficulty in breathing, If bones and joints get swollen or discolored, Intensely painful, Immovable.

Death summary should be given after death of a patient:

Format for DEATH SUMMARY

Patient Name:

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Medical Record No.:

Admitted:

Deceased:

Consultations:

Course of events:

Procedures:

Final Diagnosis: :

Immediate Cause of Death: :

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